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IN THE UNITED STATES DISTRICT COURT

FOR THE DISTRICT OF OREGON

PORTLAND DIVISION

DISABILITY RIGHTS OREGON,
METROPOLITAN PUBLIC DEFENDER
SERVICES, INC., and A.J. MADISON,

Plaintiffs,

v.

PATRICK ALLEN, in his official capacity as
head of the Oregon Health Authority, and
DOLORES MATTEUCCI, in her official
capacity as Superintendent of the Oregon State
Hospital,

Defendants.

Case No. 3:02-cv-00339-MO (Lead Case)
Case No. 3:21-cv-01637-MO (Member Case)

OCTOBER 3, 2022, REPORT TO NEUTRAL
EXPERT

JAROD BOWMAN, JOSHAWN DOUGLAS-SIMPSON,

Plaintiffs,

v.

DOLORES MATTEUCCI, Superintendent of the Oregon State Hospital, in her individual and official capacity, PATRICK ALLEN, Director of the Oregon Health Authority, in his individual and official capacity,

Defendants.

Case No. 3:21-cv-01637-MO (Member Case)

The parties in the above-captioned cases are endeavoring to reach a global resolution regarding the Oregon State Hospital's (OSH) obligations to admit those found unable to aid and assist in their defense (Aid and Assist patients) and those found guilty except for insanity (GEI patients). To achieve such a resolution, the parties are continuing to engage in regular collaborative meetings with a mutually-agreed upon neutral expert, Dr. Pinals.

Pursuant to the parties' Interim Settlement Agreement and the Court's December 21, 2021, Order Consolidating Cases and Appointing a Neutral Expert, and the Court's June 7, 2022, Order Continuing the Neutral Expert's Appointment, the attached charts provide Defendants' October 3, 2022, Progress Report setting out: (1) completed actions to achieve compliance; (2) actions planned to achieve compliant (some actions will remain continuing and not completed until compliance is achieved; others are ongoing and once completed will be moved to the completed actions chart); (3) and barriers (if any) to completing the continuing and ongoing planned actions. Acronyms used in the attached chart are as defined below.

GLOSSARY OF ACRONYMS

ACRONYM	DEFINITION
AA	Aid and Assist
AOCMHP	Association of Community Mental Health Providers
CA	Contract Administrator
CCBHC	Certified Community Behavioral Health Clinic
CCO	Coordinated Care Organization

ACRONYM	DEFINITION
CFAA	County Fiscal Assistance Agreement
CJC	Criminal Justice Commission
CLE	Continuing Legal Education
CMHP	Community Mental Health Provider
CMS	Center for Medicare & Medicaid Services
CTCLUSI	Confederated Tribes of Coos, Lower Umpqua Siuslaw Indians
CRR	Conditional release ready (PSRB/GEI population)
DAS	Department of Administrative Services
DOJ	Department of Justice
DRO	Disability Rights Oregon
FTE	Full Time Equivalent
GC	General Counsel
GEI	Guilty Except for Insanity
HLOC	Hospital Level of Care
HSD	Health Systems Division
IMPACTS	Improving People's Access to Community-Based Treatment, Supports, and Services
MOOVRs	Multi-Occupancy OSH Vacancy Resource & System Improvement Team
MPD	Multnomah Public Defenders
NWRRC	Northwest Regional Reentry Center
OAR	Oregon Administrative Rules
OC&P	Office of Contracts and Procurement
OCDLA	Oregon Criminal Defense Lawyers Association
OHA	Oregon Health Authority
OJD	Oregon Judicial Department
OPA	Operations and Policy Analyst
OSH	Oregon State Hospital
PDES	Program Design and Evaluation Services
PDTT	Person Directed Transition Team
PPS	Prospective Payer System
PSRB	Psychiatric Security Review Board
RAC	Rules Advisory Committee
RFA	Request for Application
RFGP	Request for Grand Proposal
RFP	Request for Procurement
RTP	Ready to Place
SDOH	Social Determinants of Health
SPA	Special Purpose Appropriation
SRTF	Secure Residential Treatment Facility

Mink/Bowman Oct 3rd Progress Report - Completed

	Recent and Planned Actions to Achieve Compliance	Document	Deadline	Status	Update	Barriers to Implementation
1	988 Crisis System					
2	Establish work group with 911 Regional Advisory Committee	OHA's February 2022 Progress Report		Completed - 2/24/2022	OHA just had the first meeting with the 911 PSAP group today Feb 24. 2022	
3	Accountability					
4	Admission Work					
5	Reduction in hospital admissions for individuals that can appropriately be served in other settings. In its February 3, 2022, progress report to Dr. Pinals, OHA will describe what it is currently doing to prevent or reduce the growth of hospital admissions of individuals who can be clinically served in a less medically intensive level of care (particularly aid and assist patients).	Dr. Pinals' January 2022 Report	02/03/22	Completed - 2/3/2022	Completed upon submission of "Mink & Bowman February 3 Progress Report" to Dr. Pinals	
6	Modify expedited admission processes to emphasize consultative availability upon request regardless of referral source, with protocol development in consultation with the Neutral Expert to go into effect by June 30, 2022. (I.B.7.a)	Dr. Pinals' June 2022 Report	06/30/22	Completed - 6/30/2022	Notice of modification to expedited admission process sent on 6/27/22 to stakeholder with the updated process taking effect 6/30/22.	No barriers identified at this time.
7	Community Housing Capacity					
8	Expansion of Lane county contract which funds 1-8 bed SRTF on the grounds of OSH's Junction City campus to fund a total of 3-8 bed SRTF facilities for a total increase of 16 beds above current capacity.					
9	Contract expansion signed and effective.			Completed - 4/21/2022	Contract has been signed and is currently effective.	
10	Data and Reporting					
11	Data dashboard. By March 1, 2022, the parties, with input of the neutral expert, will develop a data-dashboard with agreed-upon elements to show progress in reaching compliance and a plan for implementation, including a reporting cadence and distribution list. Anticipated distribution of this data by April 1.	Dr. Pinals' January 2022 Report				
12	HSD to meet with OSH to coordinate dashboard project.	Dr. Pinals' January 2022 Report		Completed - 2/24/2022	OSH and HSD met on 2/24/22 to discuss dashboard draft.	
13	Meet with OSH, OHA, and neutral expert to finalize data elements.	Dr. Pinals' January 2022 Report	03/01/22	Completed - 2/25/2022	Meeting occurred on 2/25/22	
14	Data dashboard presented to OJD and DRO	Dr. Pinals' January 2022 Report		Completed - 3/9/2022	Meeting occurred 3/9/22	
15	Anticipated distribute of data dashboard.	Dr. Pinals' January 2022 Report	04/01/22	Completed - 4/20/2022	The first dashboard went out on the first Monday in April (4/4/22) and future monthly dashboards will be sent out around the first Monday of each month.	
16	OHA will review whether additional staff are required to support this effort (Data Dashboard) in a sustained capacity and, if so, bring that request to the legislature. DRO/MPD will actively support that request if made.	Dr. Pinals' January 2022 Report		Completed - 3/3/2022	OSH staff can manage this work if dashboard is limited to OSH data. If dashboard expands to include community or statewide system data staffing support will be required at OHA to support this effort.	
17	Discharge Work					

18	Discharge processes. By March 1, 2022, OSH Social Work Department in coordination with OHA Office of Behavioral Health will review internal discharge processes for all commitment types with the intent to streamline multiple processes that have shown beneficial outcomes into one process with nuance between legal status clearly defined. The results of this review will be presented in MOOVRS on March 3, 2022, and will be shown to the neutral expert and the parties with the goal of implementation of new protocol by March 14. Progress on this will be summarized in the March 3, 2022, progress report to Dr. Pinals.	Dr. Pinals' January 2022 Report	03/01/22	Completed - 4/25/2022	Work has been completed and remains a living document. Follow up action includes identify all interested parties to present the Discharge map to.	
19	OSH staff will work with PDDT team to support 370 patients RTP with complex discharge needs	Dr. Pinals' January 2022 Report		Completed - 4/25/2022	PDDT criteria has been finalized to serve A&A and GEI patients, including those identified on the RTP list who meet criteria and will benefit from PDDT support to increase the likelihood of sustainable transition into the community.	
20	OHA/OSH facilitating GEI early discharge review for those reviewed ready	Dr. Pinals' January 2022 Report		Completed - 3/21/2022	Six patients identified by DOJ for readiness while in jail and subsequently admitted to OSH. All 6 were evaluated for proposed placement and 5 were accepted with subsequent Conditional Release (CR) hearings. Two were approved for CR and are now waitlisted for placement; 1 hearing was set over; 1 had placement rescinded with a 90 day set over; 1 client was denied CR and set over for 4 mos.	
21	OSH and HSD will continue to review clients for NWRRC	Dr. Pinals' January 2022 Report		Completed - 5/24/2022	Process for reviewing NWRRC referrals and providing technical assistance to NWRRC has been developed. Weekly meetings have been ended but ad-hoc meetings will be scheduled as needed.	
22	DOJ general counsel coordination in GEI cases where community evaluations were not ordered	Dr. Pinals' January 2022 Report		Completed - 5/25/2022	No longer relevant. Cases where evaluations were not ordered have been handled. In-custody GEI defendants are on same admissions list at in-custody 370s and are being admitted.	
23	By June 2022, OHA should re-establish prior policy and discharge .370 defendants back to the committing county upon a forensic evaluation of "able." For those individuals for whom an evaluator opines they are "able" OHA should stop its temporary policy established during the pandemic of keeping defendants up to 30 days past the evaluation where a party has contested that finding. This has contributed to extended lengths of stay for many individuals. Although it is understood that the official finding is based on an adjudication, for most cases as noted in the data above, when there are external evaluations, they agree with the original forensic review. (I.B.2)	Dr. Pinals' June 2022 Report	06/30/22	Completed - 6/30/2022	On June 27, 2022, OSH sent notice to identified stakeholders, informing them that on June 30, 2022, OSH will cease the contested notice process for those patients whom FES opines are "able". In other words, on June 30th OSH is resuming its prior practice of returning "able" patients to the county upon the filing of the FES evaluation.	
24	Evaluations / HLOC Reviews					
25	By July 1, 2022, OSH should develop plans for prioritization of (I.B.3)					

26	a) early referrals for evaluations of persons in Aid and Assist process at OSH to re-examine their competence to stand trial; (I.B.3)	Dr. Pinals' June 2022 Report	07/01/22	Completed - 6/22/2022	"A) The process for early referrals has been mapped out and is on the OSH intranet ("OWL"). The information on the site includes information regarding when to submit an early referral, the early referral process map, the information needed by the evaluator for the early referral, and the request for early referral form. FES evaluators have been prioritizing early referral requests, with a reminder being sent on 6/22/2022 to continue doing so with Dr. Pinals' report highlighted for them. On June 30th, OSH leadership will be sending an OSH-wide email reminding everyone at OSH that early referrals may be made; that email includes the link to the information on the OWL site. FES has been keeping track of all early referrals and ensures that evaluators prioritize early referral requests. They will continue to do so."	
27	Funding					
28	CMHP Funding					
29	CMHP Funding Increase April-June 2021	OHA's February 2022 Progress Report		Completed - 7/30/2021	CMHP's were provided an additional \$2.25m in funding through amendment to the CFAA covering the period April-June 2021.	
30	CMHP Funding Increase July-December 2021	OHA's February 2022 Progress Report		Completed - 12/30/2021	CMHP's were provided an additional \$2.25m in funding through amendment to the CFAA covering the period July-December 2021.	
31	Intensive Services Unit fully staffed	OHA's February 2022 Progress Report		Completed - 2/28/2022	The Intensive Services unit is now fully staffed. Staff are being on-boarded and have begun work on various projects to decrease OSH census and improve community engagement.	
32	Supports in the community. In its February 3, 2022, progress report to Dr. Pinals, OHA will describe the ways in which it has identified and funded additional supports in the community for people who are restored to competency, which will generate ongoing discussion with the parties regarding impacts on compliance.	Dr. Pinals' January 2022 Report	02/03/22	Completed - 2/3/2022	Completed upon submission of "Mink & Bowman February 3 Progress Report" to Dr. Pinals	
33	OHA to explain 2021 legislatively allocated funds.	Dr. Pinals' January 2022 Report		Completed - 2/3/2022	Completed upon submission of "Mink & Bowman February 3 Progress Report" to Dr. Pinals	
34	Junction City					

35	Use of the Junction City units. In accordance with the Neutral Expert preliminary recommendations and with the agreement of the parties, OSH will prioritize admissions of GEI patients to its Salem campus for assessment and placement, so that the GEI and A&A populations to be admitted from jail are waiting about the same length of time. For COVID-19 safety and protocols as well as overall speed of admission, at the time of this report, people are only admitted to the Salem campus, where the admissions monitoring units and staff are centralized. After the admission process is complete to OSH, OSH will assess for appropriate movement to Junction City placements based on all appropriate clinical and patient safety considerations, thereby opening up subsequent additional beds for admission on the admissions unit.	Dr. Pinals' January 2022 Report				
36	Utilize the opening of the additional Junction City unit to facilitate admission of patients into OSH, including GEI patients to equalize waitlist times to create the ability for a more uniform admissions protocol.	Dr. Pinals' January 2022 Report		Completed - 1/28/2022		
37	Transfer of patients to Junction City units begins	Dr. Pinals' January 2022 Report		Completed - 2/1/2022		
38	Junction City beds full			Completed - 4/20/2022	Junction city has opened their final unit and all beds have been filled.	
39	Legislative Proposals					
40	Other legislative actions. In its February 3, 2022 report to Dr. Pinals, OHA will further describe any additional relevant legislation it has proposed that may impact compliance. DRO/MPD will support OHA's legislative requests as appropriate.		02/03/22	Completed - 2/3/2022	Completed upon submission of "Mink & Bowman February 3 Progress Report" to Dr. Pinals.	
41	County financial risk sharing. OHA has proposed legislation that would allow charging counties for patients who no longer require HLOC on the OSH 9b list beyond a date threshold. DRO will support this legislative effort and/or engage with stakeholders in developing an alternative that promotes enhanced county engagement and accountability in supporting timely transitions to community.	Dr. Pinals' January 2022 Report		Completed - 3/18/2022	Neither legislation or budget note moved forward during the 2022 legislative session. Two information hearings were held before the legislature during the 2022 legislative session.	
42	SB 295 Adherence					
43	Focus on maximizing adherence to SB 295. Adherence to SB 295 (law providing for discharge of .370 patients who no longer require HLOC) is critical to make room for new admissions to OSH and to have individuals who no longer need hospital level institutional care placed in a less restrictive setting. According to tracking by the State, some state courts and CMHPs are not following this law as it is written, which results in .370 patients on the Ready to Place List staying at OSH for longer than necessary or allowed by law. To support adherence to SB 295, the parties will do the following:					

44	Advocacy. DOJ will evaluate cases on a state-wide basis for direct legal intervention on behalf of OSH where they determine that SB 295 is not being followed by state courts or CMHPs. DRO will develop, and revise as needed, an amicus brief that it will file such cases where appropriate. DOJ will notify DRO about the OSH intervention and will provide information needed for DRO to evaluate whether to submit such an amicus brief.	Dr. Pinals' January 2022 Report			Item closed out as completed due to a similar items inclusion in Dr. Pinals' June 2022 report. That recommendation will continue to be tracked with the added language.	
45	Tracking: OSH will evaluate the impact on bed space of measures taken under this section to reduce discharge wait times, and report to Dr. Pinals and DRO in currently-scheduled regular meetings.	Dr. Pinals' January 2022 Report		Completed - 4/20/2022	Data dashboard completed with first dashboard sent out on 4/4/22. OSH will utilize the data dashboard to evaluate and discuss ongoing bed space impacts with DRO and Dr. Pinals.	
46	Education. DOJ/OHA, DRO, and MPD will develop a one-hour CLE on SB 295 for defense lawyers. They will coordinate with the Oregon Criminal Defense Lawyers Association for a virtual presentation in the near future. The parties will continue to work together and consult with OJD to determine whether a similar presentation for OJD and prosecutors would also be beneficial.	Dr. Pinals' January 2022 Report		Completed - 4/26/2022	Several meetings have been held with OJD, DRO, Dr. Pinals, OHA, and DOJ re compliance with SB 295. OJD has distributed a memo to all state court judges summarizing the steps courts must take upon receipt of a ready to place notice. MPD has scheduled a CLE re SB 295 to present to OCDLA for 3/31/22. DRO and DOJ participated as co-presenters.	
47	As soon as feasible, OHA/OSH should develop a public facing Mink/Bowman website to inform stakeholders and provide public access to items such as related Federal Court Orders related to this matter, settlement agreements, public reports, legislative testimony, identified key relevant policies, funding opportunities, notices of open meetings, and the distributed data dashboard reports, and any other information that would help stakeholders understand this matter and progress toward compliance. (I.A.4)	Dr. Pinals' June 2022 Report		Completed - 9/16/2022	Website launched: https://www.oregon.gov/oha/OSH/Pages/mink.aspx	
48	By August 2022, OHA should convene key partners to review the standardized assessment process and make any final recommendations that will lead to consistent and transparent placement decisions. Rule changes shall occur as needed. Input from individuals with lived experience would be helpful in this review. (I.B.1)	Dr. Pinals' June 2022 Report	08/31/22	Completed - 8/31/2022	Met with representatives from HSD and OSH to discuss LOCUS and other various tools for statewide standardization. Overall satisfaction with LOCUS noted. However, it does not capture SUD needs or violence risk. Discussed possible need for adjunct assessments (ASAM, violence risk, etc.) in addition to LOCUS to capture a fuller picture of needs that the aid and assist population has. Met with OSH stakeholders on 9/23/22 to discuss START as violence risk assessment and it was determined that this does not accurately capture the population that OSH serves.	Magnitude of providers with cultural differences across the state. Different provider sectors using same tool (forensic, hospital, behavioral health, community-based, etc.), and staffing/funding to complete the tasks. Finding an appropriate tool to assess violence risk that everyone agrees on will be difficult.
49	CRPs are seen by the Parties as a necessary component of the AA system and alternatives to OSH and need to be strengthened. To that end, OHA should conduct an inventory of the current status of CRPs and their statewide availability across all counties and present findings of this review to the Neutral Expert and DRO and MPD by August 15, 2022. Plans to address any gaps in these services should be prioritized. (I.B.14)	Dr. Pinals' June 2022 Report	08/15/22	Completed - 8/15/2022	The task has been completed. The Parties were presented to on August 10th, 2022 and the report was delivered to Dr. Pinals and OHA on August 15th, 2022.	No barriers identified at this time.

Mink/Bowman Oct 3rd Progress Report - Current

	Recent and Planned Actions to Achieve Compliance	Item Origination Document	Status	Deadline	Required Start Date Met On	Update	Barriers to Implementation
1	988 Crisis System						
2	Establish best OAR's based on best practices in collaboration with CMHP's and peers	OHA's February 2022 Progress Report	In Progress			By end of week 9/30/22, 988 and BHCS team will submit revised draft of OAR best practices for OHA leadership review	No barriers identified at this time.
3	988 call center contracts expected Q1 of 2022	OHA's February 2022 Progress Report	In Progress			Contracts are with DOJ for review, to be executed following that review. Will reach out to DOJ for updated timeline on the review process.	No barriers identified at this time.
4	Accountability						
5	OHA restructuring of CFAA to increase accountability over the next 3 to 5 years	OHA's February 2022 Progress Report	In Progress			Due to conflicting OHA priorities meetings have been postponed until the fall. Current work that is being done right now is research, data analysis and learning, specifically on other states, the CCBHC care delivery, and the Clubhouse model used by private insurance.	-Competing BH Transformation priorities identified by leadership continues to keep this work at a slower pace than expected.
6	PDES research study underway related to Aid and Assist	OHA's February 2022 Progress Report	In Progress			Work continues by PDES to collect and analyze both qualitative and quantitative data for the report. Additional staff have been brought on to work on the qualitative data analysis. Recently the data sharing agreement that DOJ is working on between OSH and PDES was deemed to not be the correct way to transfer the data. DOJ has begun working on a different agreement that will allow for the data sharing to move forward.	Data sharing agreement between OSH and PDES.
7	In consultation with the Neutral Expert and the plaintiffs and in an ongoing manner, OHA should review existing contracts with the CCOs and CMHP's to determine the scope of the existing contractual obligations to serve the Aid and Assist and GEI population. I understand these discussions are also happening in the legislative workgroups, but a focus on this population in particular is imperative and urgent. OHA should provide monthly updates on this in its regular progress reports to the Neutral Expert. (I.B.11)	Dr. Pinals' June 2022 Report	In Progress			Intensive services unit staff continue to work on assembling the required contract information and consolidating it. Initial draft planning activities continue.	No barriers noted at this time.
8	OHA should require counties to ensure ongoing CCO enrollment for all eligible individuals who have been under an Aid and Assist order within the past two years. (II.6)	Dr. Pinals' June 2022 Report	In Progress			Item placed on hold pending decision on the 1115 waiver. Work recommence after that determination has been made.	Current barrier is waiting on the 1115 waiver. After that decision has been made the next barrier will be adjustments to the CCO contract to allow for this recommendation.
9	Benchmarks						
10	It is my recommendation that the following incremental benchmark goals be used to assess progress toward compliance in this Mink case (II.A)	Dr. Pinals' June 2022 Report					
11	Average wait time to admission 7 days or fewer	Dr. Pinals' June 2022 Report	In Progress	02/14/23		N/A	

12	Average wait time to admission 11 days or fewer	Dr. Pinals' June 2022 Report	In Progress	01/01/23		N/A	
13	Average wait time to admission 17 days or fewer	Dr. Pinals' June 2022 Report	In Progress	10/01/22		N/A	
14	Average wait time to admission 22 days or fewer	Dr. Pinals' June 2022 Report	In Progress	08/01/22		N/A	
15	Communication						
16	Beginning in the next fiscal year, OHA should produce an annual report on CRP activities for public access to inform further legislative needs for communities to best deliver CRP services, inform proposals for legislative change, resource needs, and inter-relationships of stakeholders involved with CRP participants and the courts. (II.3.c)	Dr. Pinals' June 2022 Report	Not Started	06/30/23		Work on this item was paused due to competing priorities and staffing changes. No further work has been done on this item in September.	Resource related to report writing, data collection, and SME time.
17	The State has since developed a website to provide information about behavioral health spending and improve transparency for it (See: https://www.oregon.gov/oha/HSD/AMH/Pages/index.aspx). This should continue to be updated. (II.B)	Dr. Pinals' June 2022 Report	In Progress			The website continues to be updated with relevant materials.	
18	Community Housing Capacity						
19	Expansion of Lane county contract which funds 1-8 bed SRTF on the grounds of OSH's Junction City campus to fund a total of 3-8 bed SRTF facilities for a total increase of 16 beds above current capacity.						
20	Expansion of 2-8 bed SRTF facilities open.		In Progress			Madrone Ridge is open and there are now two SRTFs open on the Junction City OSH campus. Manzanita Ridge is the 3rd cottage. According to the third-party administrator, Manzanita Ridge has linens on the beds and everything is ready to go, except staffing.	Workforce shortage is affecting the entire state and it has been a challenge to secure adequate and appropriate staffing for all of the cottages.

21	Regional development and innovation investment	OHA's February 2022 Progress Report	In Progress			- We have received most of the Final Recommendation Progress Reports from our Planning Grant partners, and we are synthesizing the data and will then analyze and format the qualitative data received. - We continue working with the Planning Grant Partner Network in developing a presentation they will have the opportunity to present to the Budget Co-chairs of the Joint Ways and Means Subcommittee, and during the next Legs. Long Session in February. - The Planning Grant Partners have requested that we continue to offer bimonthly Office Hours with them as collaboration and networking opportunities - Funded projects awarded through RFP #5340 include, funding awarded to the following community partners: Sequoia, New Narrative – Pacific, New Narrative – Mill, Live Grow Share, Columbia Care, Shangri-la, and Telecare, for a total of 69 beds. - The first round of applications pulled for the Licensed and Supportive Housing Request for Grant Applications (RFGAs) was done on 7/29, and the evaluation process is in process, with the next pull will be 8/19, 8/31, 9/30, and subsequently will be pulled monthly through 12/31/22.	No barriers noted at this time.
22	Continuing Actions						
23	HSD will continue coordination with CMHP's related to discharge and engagement expectations	Dr. Pinals' January 2022 Report	In Progress			Intensive services team continues to coordinate and meet regularly with CMHP's	No barriers identified at this time.

24	General counsel for OSH will continue already ongoing efforts to support compliance through targeted communications with individual defense lawyers and prosecutors. MPD will now also make themselves available to try and intervene with defense lawyers to ensure they follow SB 295. (I.B.9.a)	Dr. Pinals' January and June 2022 Report	In Progress			GC continues with this work and reviews a new RTP list weekly, working through each case to determine if/what violations of SB 295 are impeding OSH efforts to discharge persons who no longer need HLOC. GC continues to intervene informally, as well as refers cases to Trial for intervention.	While GC has access to some records in ecourt, some orders and reports are marked "confidential" and are inaccessible, which makes it difficult to see if there should be intervention and/or how to intervene. Municipal court records are entirely inaccessible as there is no database, such that GC must request physical copies, which are then not up to date by the time they are reviewed, again potentially impeding efforts. Finally, GC is reliant on parties and courts being willing to take different action pursuant to SB 295. Thus far, many parties/courts will still find HLOC solely based on lack of placement.
25	DOJ will continue evaluating cases on a state-wide basis for direct legal intervention on behalf of OSH where they determine that SB 295 is not being followed by state courts or CMHPs. DRO will develop, and revise as needed, an amicus brief that it will file such cases where appropriate. DOJ will notify DRO about the OSH intervention and will provide information needed for DRO to evaluate whether to intervene or submit an amicus brief. DOJ will track reasons courts are not ordering discharge. DRO will also enlist the advocacy of MPD when appropriate. (I.B.9.b)	Dr. Pinals' June 2022 Report	In Progress			As of 8/29/2022, DOJ has evaluated 98 cases for intervention. DOJ has prepared pleadings for intervention in 37 cases and has conferred with counsel in 16 cases. DOJ filed motions to intervene in 18 cases, 10 of which were granted and 4 of which were denied. In one of the cases where the motion was denied, the court nonetheless ordered a CMHP evaluation, as requested in the motion. In 2 other cases where the motion was denied, the court nonetheless stated that it would consider the SB 295 criteria as requested in the motion, and subsequently terminated commitment. In 3 cases, the defendant was discharged without the court either granting or denying the motion. In 2 cases, conferral with counsel prompted them to take steps to have the court order a Community Mental Health Program (CMHP) evaluation. One of those cases was later dismissed, and in the other case the defendant was placed on community restoration. 7 cases have been dismissed, and in 10 cases the defendant was placed on community restoration. DOJ is also identifying cases where an amicus brief from DRO might be warranted and has been discussing such cases with DRO. DRO moved to intervene in 1 case. DOJ is maintaining a tracking chart documenting in detail its efforts to intervene in these cases. Due to legal work	In some cases, circumstances change so that by the time pleadings are prepared and DOJ has conferred with counsel, intervention is no longer appropriate. Counsel sometimes opposes intervention on the grounds that they expect progress to be made shortly, even without following the SB 295 process. In one case, counsel has opposed intervention on the grounds that OSH lacks standing to intervene. Case registers do not always clearly reflect what is happening in each case, especially given that some courts do not appear to be following the standard processes for SB 295, CMHP evaluations, and findings of fitness. Some court orders are designated confidential, so DOJ cannot access them through the case register to verify whether they contain findings and orders that comply with SB 295.

26	OHA to engage with Multnomah County stakeholders to discuss the feasibility of a jail population and 9(b) review committees.	Interim Settlement Agreement	In Progress			Intensive Services team has been meeting regularly with Multnomah County stakeholders . This meeting occurs bi-weekly.	No barriers identified at this time.
27	OHA Intensive Services Unit collaboration meetings with OJD's State Court Administrator		In Progress			One meeting has taken place. Will continue to collaborate.	No barriers identified at this time.
28	OHA should continue in regular meetings with DRO and MPD and the Neutral Expert to discuss implementation of legislatively appropriated funds that have the potential to help OHA achieve compliance, to address remaining questions about prior spending decisions and to foster planning for ongoing support of the above recommendations to achieve compliance. (II.B)	Dr. Pinals' June 2022 Report	In Progress			Meetings are occurring regularly.	No barriers noted at this time.
29	Finally, the Parties should continue to meet regularly with the Neutral Expert at a cadence to be determined as recommended by the Neutral Expert and in consultation with the Parties, but no less than monthly to track progress and discuss plans for the implementation of the recommendations outlined in both the First and this Second Report of the Neutral Expert Report in this matter. (Concluding Comments)	Dr. Pinals' June 2022 Report	In Progress			Meetings are occurring regularly	No barriers noted at this time.
30	Data and Reporting						
31	By October 2023, OHA should enhance CRP data reporting from quarterly to more active regular contemporaneous reporting (and fund the needed infrastructure to do so) so that reports can be generated as needed by OHA (II.3.b)	Dr. Pinals' June 2022 Report	In Progress	10/31/23		Intensive service unit staff are in current development of a plan for the execution of this task. Meeting scheduling will begin once an initial plan is developed.	No barriers noted at this time.
32	OSH provided the first data dashboard for this matter in April 2022, and monthly thereafter. To understand trendlines, it will be helpful to produce these dashboards twice per month, and this should begin by August 2022. (I.A.1)	Dr. Pinals' June 2022 Report	In Progress	08/31/22	08/15/22	Original and additional report will continue to be sent out at the agreed upon cadence, with the agreed recipients, and with the required data.	No barriers identified at this time.
33	By the end of June 2022, OHA, DRO, and MPD should begin to engage with stakeholders to review this data and develop a process to best use this data to inform system change at local levels. (I.A.1)	Dr. Pinals' June 2022 Report	In Progress	06/30/22	06/29/22	An initial meeting took place and further coordination is happening to move forward.	Challenges in getting all stakeholders involved due to conflicting scheduling and vacations.
34	OHA/OSH should work in partnership with OJD to examine best mechanisms to share their own data and utilize regular data reports from each entity to mutually inform practices. (I.A.3)	Dr. Pinals' June 2022 Report	In Progress			Follow-up meeting held as schedule on 7/22/22. Intensive services unit has had a staffing change which has postponed work on this item. Work will continue after staffing has been decided.	No barriers identified at this time.
35	Discharge Work						
36	OHA shall amend the OARs applicable to AA Ready-to-Place defendants to clarify that the treating clinical team's clinical recommendations primarily guide discharge planning. Consistent with clinical best practice and existing legal standards regarding the ADA's integration mandate, level of care should be the least restrictive. CMHPs should provide information about what is available in the community including any reasonable options for a referral to a different community supportive placement when clinically appropriate, if the identified recommended "level" is not available. This might include, for example, providing information about a lower level of care that could be crafted with enhanced supports to meet the individual's needs. (I.B.9.c)	Dr. Pinals' June 2022 Report	Not Started			Initial discussion with team member took place in July. That team member has since gone out on extended leave. No progress was made in August due to vacation schedules across agencies. New OHA/HSD team member Michael Oyster is now assigned and an initial meeting will be scheduled with him. Much of this work is already done by OSH	Limited residential placements at all levels of care, engagement of county partners in the midst of implementing the Mossman ruling.
37	Education						
38	OHA should foster best practices in CRP through collaborative training opportunities across counties and in consultation with OJD, municipal courts, defense, and prosecution, by offering trainings/community of practice opportunities beginning by October 2023. (II.3.d)	Dr. Pinals' June 2022 Report	Not Started	10/31/23		Training will be based on product of recommendation Dr. Pinals June report II.3.a. Work will begin on training once the CRP manual is further in development.	No barriers noted at this time.

39	By July 1, 2022, plaintiffs should develop a plan presented to the Neutral Expert working in partnership with OJD and other stakeholders, and OHA should collaborate on this, to develop multidisciplinary education for defense, prosecution and judiciary regarding the importance of maximizing the use of diversion from Aid and Assist processes for any defendant for misdemeanor defendants and for those defendants for whom prosecution is not likely to be pursued. Training should also include information about accessing and prioritizing community restoration for those that cannot be diverted from criminal case processing and can meet the criteria for such services in the community. The work between DRO plaintiffs and OHA should not be construed to undercut plaintiff's independence and legally mandated role. (I.B.4)	Dr. Pinals' June 2022 Report	In Progress	07/01/22	06/29/22	The presentation content was developed through collaboration between DRO, OJD, and OHA and the presentation was scheduled. Prior to the presentation date Judge Mosman released a new federal court order which both directly impacted what was being presented and also shifted current focus to implementing the current order.	Current uncertainty and tumultuous environment related to federal court order and community implementation.
40	Evaluations / HLOC Reviews						
41	OHA/OSH should continue to support work to develop improved infrastructure and efficiencies for forensic evaluations. I recommended in my First Report the need to examine evaluation practices in greater depth. OJD has been taking the lead convening a broad stakeholder group to examine structures and funding for Oregon forensic evaluation services. With this in mind, OJD has agreed to lead in the writing of a report regarding the workgroup's efforts, and OHA/OSH and the other Parties in the Mink/Bowman matter should review and refine any drafts of that report before finalizing. This report would help inform any future recommendations or legislative proposals. (I.B.10)	Dr. Pinals' June 2022 Report	In Progress	10/31/22		Work continues on this report including collaboration and discussion at both the GAINS full Competency to Stand Trial committee and the FES subcommittee, which include representation from OHA, OJD, forensic evaluators, CMHP's, DA's, and defense. OJD is working on development of the report as they refine recommendations on potential changes to the current system or implementation of a new system for forensic evaluations.	No barriers to report completion. Implementation of considerations will require legislative support and funding.
42	For now, those individuals opined as "never able," or "med never" should be further studied for potential process change to support direct community discharge, if clinically appropriate, with CMHP assistance rather than routing back to jail. This will require further discussion. (I.B.2) Although beyond these legislative recommendations, there should be further exploration of opportunities for defendants found Unable to Aid and Assist or "Med Never" to ensure access to appropriate services as needed. (II.2.e)	Dr. Pinals' June 2022 Report	In Progress			Staffing being looked to see who should be heading up work on this item, more details to be provided once details have been worked out.	No barriers identified at this time.
43	By July 1, 2022, OSH should develop plans for prioritization of (A has been completed and moved to "Completed" tracker) and (B see below) (I.B.3)						

44	b) earlier reviews for Hospital Level of Care (HLOC) determinations for AA patients at OSH to clinically determine readiness for stepdown or discharge as early as possible, with implementation of both these plans as expeditiously as possible. (I.B.3)	Dr. Pinals' June 2022 Report	In Progress	07/01/22	06/29/22	OSH and OHA met with Dr. Pinals on 9/13/22 to discuss OSHs determination that HLOC assessments will be completed in coordination with the 10-day Treatment Care Plan meeting. For patients not needing HLOC, OSH will complete a LOCUS at 10 days and file notice with the court and parties that the patient no longer needs HLOC. Process is repeated every 30 days thereafter.	Dr. Pinals' recommendation is to complete assessment and filing within a 48 or 72 hour timeframe. OSH barriers include significant vacancies in necessary staff and additional workload related to CMS Plan of Correction and increase in admissions/discharges following the Mosman ruling, which make an additional IDT meeting (not otherwise required by CMS/TJC standards) difficult to implement without impacting other clinical care. OSH has also not yet determined if all elements of the HLOC can be answered with confidence in a 48-72h time frame.
45	OHA should present a plan to ensure that community evaluations are scheduled within 15 days of receipt of the order and completed within 45 days. This may include review of data on the timeliness of these evaluations, modification of contracts, rules, or other means to accomplish this recommendation. OHA should take all reasonable steps to implement such a plan and secure funding needed to implement it. OHA, in consultation with the PSRB, should present to the Neutral Expert a proposed timeline for implementing this plan as expeditiously as possible.	Dr. Pinals' June 2022 Report	In Progress			OHA and PSRB have held several meetings on the topic of reporting timeframes and mechanisms to expand funding. Recurring appointments have been set between stakeholders with a cadence of every week. A timeframe has not been firmly discussed for the proposed changes as there are a variety of solutions being explored including changing service element language or a change in rules. Work continues and is a regular topic of discussion between staff at OHA and PSRB.	No barriers identified at this time.
46	Funding						
47	IMPACTS Grant Program	OHA's February 2022 Progress Report	In Progress			All 4 new project proposals have been funded.	No barriers identified at this time.
48	CMHP Funding						
49	CMHP RFA FY 2023	OHA's February 2022 Progress Report	In Progress			10 of the now 24 contracts are fully executed. A contract for Lincoln County has been drafted and submitted to OC&P for processing.	No barriers identified at this time.

50	CCBHC	OHA's February 2022 Progress Report	In Progress			The CCBHC team has completed all 12 site reviews for programmatic fidelity and regulatory compliance. Next steps include comprehensive reporting which will set revised recommendations of Oregon requirements for the CCBHC model, developing an official certification process, as well as feedback to individual clinics on compliance performance. The team has also initiated 1) a learning collaborative for existing CCBHC's to develop technical assistance that will be shared with pilot clinics if the program is expanded statewide, and 2) a consumer-driven steering committee that will inform OHA's advice to legislative process re: CCBHC. The project director is set to meet with the CCBHC legislative workgroup again in October and November. Because model legislation drafted thus far aims to make CCBHC statewide in statute, the team will also begin developing a call for proposals to keep competition open at this time (unless directed otherwise by executive sponsorship). The CCBHC evaluation is set to present pre-liminary findings mid- to late October to OHA.	Barriers noted in August carried over into September, including that CMS has not announced the mechanism for expansion for demonstration states. If legislative session in the Spring aims to take CCBHC statewide, the difference between this potential mechanism, which includes an enhanced match, and a SPA (no enhanced match) could be tens of millions or more.
51	OHA should explore all available means to provide additional resources for community providers to prepare timely discharge plan development for GEI patients including evaluations by CMHPs. This will include devising a funding mechanism to pay for evaluations by CMHPs as ordered by the PSRB. This may include a base rate for completing evaluations within 30 days.	Dr. Pinals' June 2022 Report	In Progress			OHA and PSRB have held several meetings on the topic of reporting timeframes and mechanisms to expand funding. Recurring appointments have been set between stakeholders with a cadence of every week. A timeframe has not been firmly discussed for the proposed changes as there are a variety of solutions being explored including changing service element language or a change in rules. Work continues and is a regular topic of discussion between staff at OHA and PSRB.	No barriers identified at this time.
52	Legislative Proposals						
53	Level of Care/Placement Decisions						
54	Starting in June 2022, OHA/OSH will work with consultation of the Neutral Expert to develop standardized assessment processes that support level of care determinations without over-reliance on a single score. (I.B.1)	Dr. Pinals' June 2022 Report	In Progress	06/30/22	06/24/22	To date we have had one meeting with the Neutral Expert to discuss the development of a standardized assessment process. This meeting was on 6/24/22. There has not been a second meeting scheduled due to scheduling difficulties. Efforts continue being made to this end.	Scheduling
55	Other Actions						

56	By October 1, 2023, OHA should, in consultation with Plaintiffs and other stakeholders, and with input from the Neutral Expert, develop a CRP manual, delineate best practices across regions, engage in training, develop standard court forms. Oregon's rules (309-088-0115) already identify elements of restoration services, but some may be ambiguous or appear crossover with other funded behavioral health services (II.3.a)	Dr. Pinals' June 2022 Report	In Progress	10/01/23		Basic research to review other community restoration programs has been done, and research will continue to this end.	Staff resources, SME time, and community bandwidth to have workgroups related to development of a standardized CRP program across the state.
57	By August 2022 regardless of whether there is a court-led "jail review", OHA should engage stakeholders to develop a process for real-time ongoing local in-jail review/consultation of all currently detained defendants in the Aid and Assist process ordered for restoration, and leverage resources expended on jail diversion programs in the community to conduct these reviews. Contract changes may be needed to fulfill this recommendation. If resources are not available for such a process, OHA and OSH should seek support for such resources. (I.B.7.c)	Dr. Pinals' June 2022 Report	In Progress	08/31/22	08/19/22	Working to communicate with community partners and judicial department regarding jail review initiative.	No barriers identified at this time.
58	Starting in July 2022, support OJD's efforts to expand the Multnomah County "jail review" initiative and prioritize AA assessments and reviews of individuals in jail who have appeared to have positive changes in mental status that would likely yield a finding of Able to Aid and Assist prior to admission to OSH and/or be eligible for diversion from the pool of individuals waiting for admission. (I.B.7.b)	Dr. Pinals' June 2022 Report	In Progress	07/31/22	06/30/22	Working to communicate with community partners and judicial department regarding jail review initiative	No barriers identified at this time.
59	By July 1 2022, OHA, OSH and the Office of Developmental Disability Services (ODDS) should meet to review the Mink/Bowman case to determine where there may be needed improvements for timely discharge from OSH and diversion for individuals with IDD in the Aid and Assist and GEI processes to appropriate community alternatives in lieu of OSH admission where feasible, as well as explore community restoration support opportunities for clients eligible for DD services.	Dr. Pinals' June 2022 Report	In Progress	07/01/22	06/24/22	We have had several meetings with ODHS and have developed a number of positive steps toward ensuring communication and coordination of systems related to discharge for both Aid and Assist as well as GEI. Much less progress has been made about possible diversionary efforts using ODHS resources as this is often needed to be done on a case-by-case basis as needs vary between individuals. Turnover of ODHS staff has resulted in some slide back in efforts, but a much more clear process for identification of eligibility has been established which has helped to identify resources for some individuals as they near discharge from OSH. Progress continues and discussions will need to be continued related to diversions from OSH admission using ODHS partners. ISD staff have had two meetings with Marion County's mental health and DD case manager and supervisor to discuss funding of PSRB related DD services and ensuring that funding is appropriate for the services needed. Several possible changes to security rates have been explored and will be discussed by OHA's Rate Review Committee.	County differences in responsiveness have created disparate outcomes; however, some of that has been resolved with new contacts within ODHS. In terms of diversion, quickly obtaining adequate records for a determination of eligibility (or redetermination) is a barrier.
60	To help offset this risk of recidivism and sustain compliance, OHA in coordination with stakeholders and in consultation with the Neutral Expert should develop a model to create "community navigators" to support individuals sent for restoration as they transition from OSH into community settings. (I.B.6) See Report for more details	Dr. Pinals' June 2022 Report					

61	Expected design for a pilot model by December 2022, which will then inform a mechanism for establishment of the pilot implementation date.	Dr. Pinals' June 2022 Report	In Progress	12/31/22		Cancelled a meeting with Neutral Expert pending more follow up from leadership, will need to be reschedule once we know more information.	Funding source and stability.
62	Model development should begin in July 2022 with	Dr. Pinals' June 2022 Report	In Progress	07/31/22	07/13/22	Cancelled a meeting with Neutral Expert pending more follow up from leadership, will need to be reschedule once we know more information.	Funding source and stability.
63	The OHA Medicaid team will continue working on the 1115 waiver, which would continue limited Medicaid coverage and for individuals at OSH under .370 orders 6 months prior to discharge. If the waiver is accepted, OHA will amend the CCO contract in 2023 to require Intensive Care Coordination for all clients currently at OSH under 370 orders in preparation for community placement. Should this occur, such ICC should be coordinated and take into account the Community Navigators, and OHA should evaluate whether the new ICC services or other available programs (such as ACT Teams) are sufficient to perform the desired functions of Community Navigators. (I.B.12)	Dr. Pinals' June 2022 Report	In Progress			Item placed on hold pending decision on the 1115 waiver. Work recommence after that determination has been made.	Current barrier is waiting on the 1115 waiver. After that decision has been made the next barrier will be adjustments to the CCO contract to allow for this recommendation.
64	Substance Use Treatment						
65	Expand access to substance use treatment including medications for addiction treatment (MAT) and contingency management (to address stimulant use disorder) in residential and community programs that serve people under AA orders. These services should be incorporated into the refinements of services offered for people in Community Restoration Programs (CRPs). (I.B.13)	Dr. Pinals' June 2022 Report	In Progress			Scheduling meeting with OSH to get more information about their assessments, reporting, and referral process for SUD tx.	Clarification on what provider certification can complete an ASAM assessment in Oregon, what facility certifications are needed for MAT prescriber, identifying other substance use assessments.
66	Similarly for the OSH population, efforts should be made to fostering greater focus on substance use treatment services for individuals in AA and GEI processes. (I.B.13)	Dr. Pinals' June 2022 Report	In Progress			OSH has been able to restart our comingled treatment mall that had been paused since March 2020 due to COVID risk. This allows OSH to provide group treatment that is accessible to patients on three units at a time rather than to only one unit at a time and will allow for some gains in efficiency of current resources. This change may provide some increase in access to substance use treatment services with current resources.	Low staffing (e.g., 34% vacancy rate in OSH Psychology), need for comprehensive training and/or recruitment of clinicians trained in SUD treatment, lack of adequate positions authority to expand services without negatively impacting other needed mental health services, OSH current focus on CMS response, increased admission workload for clinicians with changing EQJs related to Judge Mossman's order. Additional work is needed to identify needs. As we are able to stabilize staffing resources, we will develop workgroups to further develop plans for expanding services.

67	In my First Neutral Expert report, I made recommendations that were supported by the Parties regarding county financial risk sharing. Currently fiscal responsibility for utilization of OSH beds lies entirely with the state and not the counties. A greater shared focus with local entities on this resource could potentially improve its utilization management for communities, which would help increase access to more populations. I recommend shifting the January 2022 Neutral Expert Report recommendations to include incentives to the proposed cost sharing a program with CMHP and further recommend that counties, and CCOs share in both the risk and incentives aimed at reducing length of stay for individuals in the AA and GEI processes on the ready to place list. The parties should work with appropriate legislators and others to help develop formulas for this activity in advance of the 2023 legislative session. (II.1)	Dr. Pinals' June 2022 Report	In Progress	06/30/23		OHA and government relations involved in conversations weighing the benefits/lift behind going forward with this ask. Need to make high-level decision on whether this is worth the risk to all other legislative goals to include this in the next legislative period.	Stakeholder strong opposition to contract changes and legislative proposals to change this; if cost sharing shifts to include counties, counties will look for OSH/OHA to increase financial support
68	OSH staffing. OSH has submitted a request to the legislature for increased position authority and funding. DRO will actively support this legislative request. OSH submitted staffing request in the HB 5024 budget report	Dr. Pinals' January 2022 Report	In Progress			As of August 31st, OSH has submitted the Agency Request Budget that contains "sustainable Staffing Plan" position request.	Barriers include staffing needs continue to change based on new processes/policies/judgements from where we were when the "sustainable staffing plan" was developed. For instance the recent ruling by Judge Mosman will have substantial impact on staffing needs but no vehicle to address changed staffing needs. Additionally CMS compliance is another example.
69	OHA also proposed a provider rate increase to stabilize existing behavioral health providers and encourage them to expand capacity.	Dr. Pinals' January 2022 Report	In Progress			Staffing being looked to see who should be heading up work on this item, more details to be provided once details have been worked out. Anytime a change to the Medicaid State Plan is proposed OHA is required to allow for public comment. The public comment period for these rates ended Sep. 7, 2022, 11:59 p.m. Pacific Time. Since that time OHA has responded to public comments and held a webinar on the feedback. OHA submitted the Medicaid State Plan amendment (SPA) to CMS August 15th. CMS has 90 days to review and approve the SPA, At this time OHA does not anticipate a denial by CMS to the proposed rates. OHA has and will continue to provide notice, guidance, and system update timelines once CMS approval is granted. Providers will then be able to retroactively bill or rebill for services rendered after July 1st and receive the new rates.	Final approval of OHA's proposed rates are at the discretion of the Federal Centers for Centers for Medicare & Medicaid Services (CMS). OHA does not anticipate a denial by CMS to the proposed rates.

70	For Oregon, I recommend the parties work jointly with willing stakeholders to propose new legislation that decreases the maximum restoration time limits. In my opinion, Oregon's processes could be improved and access to services gained if the law aligns more with clinical data, case law, and legislation in other states that focus restoration on its intended purpose. (See Report for Specific Recommendations) (II.2.a)	Dr. Pinals' June 2022 Report	In Progress			Legislation is drafted, anticipate it will go forward with hearings in the coming legislative period. Look to continue communicating with outside stakeholders and legislators outside of legislative period regarding the need for this legislation	DAs are already lobbying against this legislation
71	In my opinion, the Court in making its findings should rely upon clinical opinions, and the forensic evaluators in rendering their opinions of restorability should provide compelling clinical data to support a substantial likelihood beyond probability that the defendant shall regain their capacity to aid and assist at the end of the restoration period, based on evidence that with treatment, the defendant is likely to evidence signs such as, symptom attenuation related to thought, mood, orientation or memory, capacity to recognize reality, reasoning, judgment and/or behavior, or improvements in functional and adaptive deficits related to being able to Aid and Assist. Evaluators should take into account the nature of the condition rendering the defendant unable to Aid and Assist with regard to restorability. For example, restorability opinions and adjudications should require specific data to support substantial likelihood beyond probability especially for defendants whose ability to aid and assist is less likely to be restored timely through inpatient or community-based restoration services such as those defendants who have repeatedly been previously found unrestorable or who have neurocognitive disorders (such as dementia and traumatic brain injury) or significant neurodevelopmental disorders contributing to their inability to aid and assist. Clinical evidence of symptom attenuation or improvements in competency functional deficits should be noted for defendants charged with a serious felony for whom an extension of the restoration period is granted. (II.2.b)	Dr. Pinals' June 2022 Report	In Progress			This item will require discussion among evaluators and key stakeholders. OSH Forensic Evaluator Certification Program Coordinator will include this item as a discussion point for the Rules Advisory Committee that will be reviewing changes to the OARs that govern forensic evaluators. FES leadership will discuss this item with evaluators in the OSH Forensic Evaluation Service. To some extent, these recommendations are tied to Dr. Pinals' recommendations regarding shortening the length of time that defendants have to be restored to the ability to aid and assist. Committee will meet in October to discuss agenda item.	N/A
72	Restoration across multiple charges should be consolidated and contiguous consecutive periods of restoration should be eliminated unless there are new charges incurred after an initial period of restoration has ended. (II.2.c)	Dr. Pinals' June 2022 Report	In Progress			Legislation is drafted, anticipate it will go forward with hearings in the coming legislative period. Look to continue communicating with outside stakeholders and legislators outside of legislative period regarding the need for this legislation	DA and municipal courts will oppose, but enough support from other stakeholders such as defense attorneys
73	Aid and Assist progress/periodic Aid and Assist reports should be brief, relying on more complete evaluations that were made for the initial findings of a defendant being Unable to Aid and Assist. The brief periodic update reports should be done at intervals, with the first three at no longer than 30, 60 and 90 days, followed by every 90 days. Aid and Assist progress updates should also be filed as soon as feasible after there is clinical data supporting that the defendant has regained abilities to Aid and Assist, and no later than the external time limits. Again, these updated reports should be brief and delineate whether there have been any changes in mental status or clinical functioning related to Aid and Assist capacities (rather than complete de novo evaluations), sufficient for the Court to make a finding as needed. (II.2.d)	Dr. Pinals' June 2022 Report	In Progress			Legislation is drafted, anticipate it will go forward with hearings in the coming legislative period. Look to continue communicating with outside stakeholders and legislators outside of legislative period regarding the need for this legislation.	Need more evaluators, staffing limited by current funding, would need to be expanded as part of legislative period.

74	Separate and distinct from staffing recommended in the January report, and recognizing that many of the recommendations throughout this report will require additional staffing, I support a proposal from OHA that they would explore all available means to obtain funding for seek one OSH data analyst and two OSH data integration specialist positions to support Mink/Bowman treatment discharge approaches, community connections, and data reporting. (II.5)	Dr. Pinals' June 2022 Report	In Progress			As of August 31st, OSH has submitted the Agency Request Budget that contains "sustainable Staffing Plan" position request.	Barriers include staffing needs continue to change based on new processes/policies/judgements from where we were when the "sustainable staffing plan" was developed. For instance the recent ruling by Judge Mosman will have substantial impact on staffing needs but no vehicle to address changed staffing needs. Additionally CMS compliance is another example.
75	If expansion of data development is needed OHA should seek to add an additional Data Technician, through the development of a Policy Option Package (POP) to be submitted to the legislature or any means available. (I.A.2)	Dr. Pinals' June 2022 Report	In Progress			Policy Option Package has been submitted. Legislative session starts in January.	Current hiring restrictions, budget
76	With regard to defendants charged with misdemeanors in the AA process, OHA/OJD/DRO/MPD should make every effort to work collaboratively with stakeholders to identify alternatives that no longer utilize OSH when there is no real Government interest in pursuing prosecution and work to pursue avenues for alternative community plans for these individuals. (II.4)	Dr. Pinals' June 2022 Report	In Progress			Staffing being looked to see who should be heading up work on this item, more details to be provided once details have been worked out. Current coordination and drafting of legislative concepts for the 2023 session with OHA leadership, legislative staff, and the governor's office.	